

SimplePractice Response to HHS RFI: Accelerating AI Adoption in Clinical Care

Submitted to: Office of the National Coordinator for Health Information Technology (ASTP/ONC), U.S. Department of Health and Human Services

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About SimplePractice

SimplePractice is a leading practice management and electronic health record (EHR) platform built for independent behavioral health, mental health, and wellness practitioners. We serve over 250,000 mental health practitioners across all 50 states, the majority of whom are solo or small-group practitioners, including licensed psychologists, social workers, counselors, marriage and family therapists, nurse practitioners, and psychiatrists.

We occupy a unique position in the AI conversation: we are both building AI-powered tools for clinical care and supporting the independent practitioners who adopt them. That gives us direct, ground-level insight into what AI adoption looks like in behavioral health settings—the wins, the hesitations, and the real-world gaps that federal policy can help close.

This submission is grounded in responses from our Clinical Advisory Board and Customer Advisory Board, many of whom use our platform daily and agreed to share their perspectives for this submission.

What We Know: AI Is Working—And Mental Health Practitioners Have Real Concerns

Where It's Working

AI-assisted documentation is already changing practice for the better. As one SimplePractice Customer Advisory Board member put it:

"Documentation currently has improved efficiency in my practice and allowed more eye contact and interactions with the patient [versus the] computer."

- Zachary Moraity, MD

One of SimplePractice's Clinical Advisors, who also uses SimplePractice's AI powered note taker with his patients, stated:

"AI assisted note taking has been a game changer, allowing me to focus on being in the moment rather than writing things down. It's actually changed the way I speak to patients. I now summarize my thoughts at the end of the session, telling patients clearly

what my impression is and plan going forward. I started doing this to make the ambient AI tool work better and I found that it really helps the patient understand how I'm thinking and forces me to explain/answer questions about my formulation."

- Ish Bhalla, MD

This reflects what we see across our platform: when AI tools are well-designed and well-implemented, they reduce the administrative burden that pulls practitioners away from their patients. That is a win worth protecting—and building on.

"One of the most exciting prospects is using AI to improve clinical care. I love the idea of AI helping develop a case conceptualization and offering treatment suggestions. I strongly believe that it would still be up to the therapist to use their background, training, and clinical intuition to modify, adapt, and employ the AI suggestions. However, having AI as a tool to assist with some of the heavy lifting would take a major administrative load off of therapists and allow them to do their jobs more effectively."

- Max Maisel, PhD

Where Mental Health Practitioners Are Hesitating

Adoption is not universal. Mental health practitioners are navigating genuine uncertainty, and that uncertainty is suppressing uptake in ways that policy can directly address. Our team and our advisors identified five core concerns:

1. Unclear liability:

When an AI-assisted note contributes to a clinical decision or a payer dispute, or a breach, who is responsible? The practitioner? The platform? The AI vendor? This ambiguity is one of the top concerns our advisors raised.

"The biggest barriers I anticipate are liability ambiguity and uncertainty around HIPAA compliance. It's hard to feel confident adopting a new tool if it is unclear who is accountable if something goes wrong."

- Moe Ari Brown, LMFT

Similarly, as we see increases in the number of AI vendors in the field, clinicians are not always clear on the standards that have been adhered to within the product.

"There's still uncertainty around how data is processed across platforms, and whether information is retained or shared beyond its intended use. I'm cautious about how patient information is handled, stored, and protected. Until HIPPA safeguards are crystal clear, caution isn't resistance; it's my ethical responsibility."

- Dr. Dwayne Buckingham

2. The learning curve:

Mental health practitioners will adopt tools that save time, but only if those tools are intuitive and come with accessible training.

"Cost is a real barrier that I have encountered in regards to adopting AI tools in my practice. Quality AI tools aren't cheap, and implementing them responsibly requires time,

training, and ongoing oversight. While I understand the benefits of AI, I am slightly concerned about the cost of adopting it into my practice.”

- Dwayne Buckingham, PhD, LCSW-C

3. Reimbursement risk:

Practitioners see very real potential value of AI in supporting reimbursement. Dr. Dwayne Buckingham states:

“I believe AI could significantly reduce my administrative burden...eligibility checks, claims follow-ups, and reimbursement processes often require additional staff many private practices can’t afford.”

However, they are also beginning to worry that AI-assisted documentation could work against them, if their notes look "too clean" or insufficiently individualized, or if patient symptoms are not accurately reflected with AI usage that payers will deny claims. Dr. Ish Bhalla adds:

“I don’t want third parties to have access to transcripts or recordings of sessions with patients for the purpose of malpractice claims or payment restrictions. I don’t want clients to feel concerned about their words and actions being used against them in any way either.”

4. Clinical integrity:

Several advisors worried about the risk of going on "autopilot", and becoming so reliant on AI that they miss a critical clinical signal. This isn't an argument against AI. It's an argument for implementation standards that keep clinical judgment front and center. One of our customer advisors, Dr. Max Maisel, notes that:

“One concern would be if therapists over-rely on AI and don't do their due diligence. However, with proper training and education about how AI works and how it doesn't work, I believe this can be largely avoided.”

5. Patient-facing AI:

Mental health practitioners want AI help with patient communication, but they're cautious. The concern: "patient frustration or AI providing inaccurate interpretation/recommendations without safeguards." Patient-facing AI features deserve a higher bar of oversight, and guidance should reflect that distinction.

Our Recommendations

The RFI asks about regulation, reimbursement, and research and development. Our recommendations are organized around those three themes, with a focus on what matters most for behavioral health and independent practice.

1. Regulation: Clarify Accountability (Responds to RFI Q1, Q2, Q3)

The problem: Practitioners don't know how existing rules apply to AI. Does using AI documentation create a HIPAA risk? What attestation is required? Who is liable if something

goes wrong? This uncertainty is a real adoption barrier, and a clear, plain-language framework from HHS could help to clarify these issues.

Specific Recommendations:

- Publish clear guidance clarifying how HIPAA, professional licensure standards, and existing documentation requirements apply when mental health practitioners use AI-assisted tools.
- Clarify the liability allocation when AI tools contribute to a clinical note, a care recommendation, or a documentation error.
- Create a certification or safe harbor pathway for EHR and practice management platforms that implement AI responsibly so practitioners can trust the tools they use.
- Establish a minimum standard for clinical attestation: AI may assist, but the practitioner's review and signature must remain the standard of record.

2. Regulation: Risk-Based Frameworks Over Bans (Responds to RFI Q3, Q5)

The problem: The emergence of consumer-facing AI chatbots that purport to provide mental health support has raised legitimate safety concerns. Some of these tools operate outside any clinical relationship, without human oversight, and without accountability for adverse outcomes, leading some policymakers to propose outright bans on "therapy chatbots." However, SimplePractice urges HHS to advocate for a risk-based framework rather than categorical prohibitions. The distinction that matters is not whether AI is involved, but whether a licensed clinician remains accountable for the patient's care and what safeguards have been put into place. The current draft notes concerns about patient-facing AI providing "inaccurate interpretation/recommendations without safeguards." This concern is well-founded, but the solution is appropriate safeguards, not prohibition of all AI in patient communication.

Specific Recommendations:

- Adopt a risk-tiered regulatory posture that distinguishes between AI tools that support practitioner judgment (lower risk) and AI tools that substitute for it (higher risk).
 - Low Risk: AI-assisted documentation, scheduling, billing automation
 - Medium Risk: AI-generated treatment suggestions, clinical decision support
 - High Risk: Patient-facing AI providing clinical guidance or interaction with practitioner involvement
 - Critical Risk: Autonomous AI making treatment decisions without clinical oversight
- Safeguards should align with the risk-tiered levels. While low risk tools may simply require practitioner attestation or platform certifications, tools with increased risk may involve a combination of safeguards appropriate for the risk level such as: human-in-the-loop reviews, patient transparency, robust safety and effectiveness validations, requirements for disclosure of limitations, scope limitations, and/or crisis protocols with mandatory clinician escalation pathways

- Avoid regulatory frameworks that treat all "AI in mental health" as a single category. AI-assisted documentation and autonomous therapy chatbots present fundamentally different risk profiles and should be regulated differently.

3. Reimbursement: Don't Penalize Efficiency (Responds to RFI Q2, Q9)

The problem: The payer relationship is a significant source of anxiety for mental health practitioners considering AI tools. If using AI to write notes efficiently is interpreted as cutting corners, rather than working smarter, adoption will stall.

Specific Recommendations:

- Issue guidance clarifying that AI-assisted documentation does not inherently constitute fraud, waste, or abuse under Medicare/Medicaid billing rules, provided clinical review is documented.
- Work with CMS to confirm that AI-assisted clinical encounters remain fully reimbursable when human judgment is clearly applied and documented.
- Explore reduced administrative burden (e.g., prior authorization simplification) for practices using certified AI tools that demonstrably improve documentation quality and access to care.
- Consider a demonstration program that tests reimbursement incentives for small and solo behavioral health practices adopting AI tools.

4. Reimbursement: Opportunity to Accelerate Adoption (Responds to RFI Q2)

The problem: The current draft appropriately raises concerns about AI-assisted documentation being penalized by payers. *But reimbursement policy is not only a risk to mitigate; it is a lever to accelerate adoption.* No CPT codes exist specifically for AI-assisted clinical activities, leaving practitioners uncertain how to document and bill for encounters where AI played a supporting role. Medicare and Medicaid documentation requirements were designed for a pre-AI world. AI could enable more efficient documentation that meets clinical needs, but current payer expectations create friction.

Specific Recommendations:

- Work with CMS and the AMA CPT Editorial Panel to evaluate whether new codes or modifiers are appropriate for AI-assisted behavioral health services, particularly for:
 - AI-supported outcome measurement (e.g., automated symptom tracking and measurement)
 - AI-assisted care coordination (e.g., summarizing patient history for referrals, matching)
 - AI-enhanced documentation (e.g., ambient listening, documentation generation)
 - AI-assisted client facing tools monitored by the clinician
- Ensure that any AI-related reimbursement policies apply equally to independent practitioners and health systems. Small practices should not face higher barriers to payment for using the same tools available to large organizations.

5. Research and Development: Prioritize Behavioral Health (Responds to RFI Q6, Q9, Q10)

The problem: Behavioral health is chronically underrepresented in health AI research, despite having one of the highest documentation burdens and one of the most significant workforce shortages in the sector. This is a gap HHS can help fill.

Specific Recommendations:

- Fund research specifically on AI use in behavioral health and mental health settings, including solo and small-group practices, not just large health systems.
- Prioritize research on patient experience and safety in AI-assisted behavioral health care, including patient-facing communication tools.
- Invest in implementation science: what training, workflows, and design patterns lead to safe, sustainable AI adoption in independent practice?
- Support public-private partnerships that bring together EHR vendors, mental health practitioners, and payers to develop behavioral-health-specific AI use standards.

Innovation in the application of AI in mental health can bring very tangible clinical value to providers, but to do so effectively, research in the application of tools in independent solo and small group practice settings is essential.

I hope AI can help me see blind spots in my clinical decision-making. It would be great to get recommendations on tests, medications, and psychotherapy interventions that I might have missed because of unconscious bias or new research that I might not know about.

- Ish Bhalla, MD

6. State Regulatory Fragmentation: A Hidden Barrier to AI Adoption (Responds to RFI Q1, Q2, Q3)

The problem: SimplePractice serves practitioners licensed in all 50 states. The inconsistent patchwork of state-level rules governing AI use is a significant barrier to use. Consider the practitioner who is licensed and providing treatment across multiple state lines. Each state may have different requirements for AI use across patient consent or disclosure, documentation standards, and/or strict use limitations. Some states have enacted restrictions on AI in health care that may sweep too broadly. While we share concerns, blanket prohibitions on AI-assisted tools risk cutting practitioners off from tools that support their practice.

This fragmentation creates compliance uncertainty that disproportionately burdens small practices. Large health systems have compliance departments to navigate multi-state requirements; solo practitioners do not.

Specific Recommendations:

- Issue federal guidance affirming that AI-assisted documentation tools used by licensed practitioners within the scope of their clinical judgment do not require additional state-level authorization beyond existing telehealth and practice act frameworks.
- Work with state licensing boards through organizations like the Federation of State Medical Boards (FSMB), the American Association of State Counseling Boards, and the Association of State and Provincial Psychology Boards (ASPPB) to develop model guidelines for AI use in behavioral health that states can adopt, reducing fragmentation.

A Note on Behavioral Health

Behavioral health is often missing from health AI conversations, but it shouldn't be. The workforce shortage is still severe in spite of mental health parity laws, the documentation burden is among the highest in any specialty, and the therapeutic relationship itself is a clinical instrument that AI can either support or undermine, depending on how it's deployed.

One dimension of this challenge is worth naming explicitly. As consumer AI tools have become widely accessible, more people are using them as a first—and sometimes only—point of contact for mental health concerns. For individuals experiencing suicidal ideation, acute paranoia, or other conditions requiring clinical judgment, this gap carries real risk: a well-designed AI can be responsive and even helpful, but it cannot assess imminent safety, coordinate care, or bear professional accountability for what happens next. The question for HHS is not whether to stop this, as it is already occurring at scale, but whether federal policy will create the conditions under which a human mental health practitioner remains meaningfully in the loop. Supporting responsible AI adoption within clinical relationships is one of the most direct levers available.

SimplePractice is committed to getting this right. We believe AI can expand access to behavioral health care, reduce practitioner and clinician burnout, and improve outcomes if the policy environment gives practitioners and platform builders the clarity and confidence to move forward.

We look forward to working with HHS to make this a reality.

“My main concern is maintaining a high standard of clinician integrity when using AI... As more tools appear on the market to help us make our practices more efficient and successful, I'm excited about that support. However, I urge clinicians to use the technology as operational support that helps them deepen therapy work.”

- Moe Ari Brown, LMFT